

Ethiopian Health Sector Transformation Program (HSTP)

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HSDP to HSTP



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Lessons for HSTP III



HSDP to HSTP

- The Ethiopian health sector transformation program was preceded by Health sector development program which had four consecutive programs.
 - **HSDP I** (1997-2002): District health offices & National Health Account
 - **HSDP II** (2002-2005): Health extension workers & immunization
 - **HSDP III** (2005-2010): HIV/TB/malaria integration & maternal services
 - **HSDP IV** (2010-2015): Quality improvement & financing
- HSTP was prepared after conducting thorough review of prior HSDP's

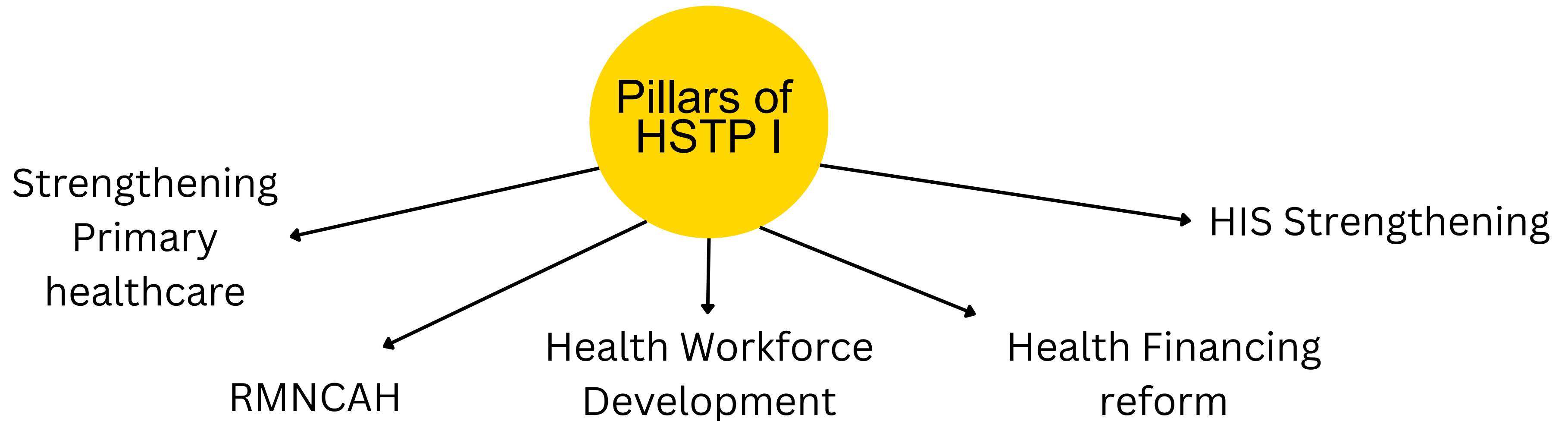
HSTP I

- PILLARS
- IMPLEMENTATION
- CHALLENGES
- LESSONS



HSTP I

- Timeline: 2015/16–2019/20
- Aim: Universal Health Coverage (UHC)
- It had 5 major strategic pillars...



1. Strengthening primary health care

- HSDP concentrated on increasing the number of healthcare centers whereas HSTP worked on their number and quality.
 - Construction: 3,000 health centers & 300 hospitals
 - 38,000+ Health extension workers deployed
 - Focus on preventive services

2. Reproductive, Maternal, Neonatal, Child, Adolescent and Youth Health (RMNCAH)

- Institutional deliveries promoted
- HPV and expanded immunizations
- Adolescent health in schools



3. Health Workforce Development

- New training institutions
- CPD implementation
- Strategic placements & incentives



4. Health Financing Reform

- Establishment of Community based healthcare insurance covering 700+ woredas.
- Introduction of health facility revenue retention and autonomy mechanisms.

5. Strengthening the Health Information system

- The Health Management Information System (HMIS) was digitized and streamlined.
- Introduction of the District Health Information Software (DHIS2) to standardize reporting.
- Emphasis on improving data quality, completeness, and timely reporting.

HSTP I Performance

- HSTP I was successful in driving substantial improvements in health outcomes, access and systems performance.

In progressive health outcomes	Service Coverage Gains	Alignment with SDG's
• MMR: 420 → 401 • U5MR: 64 → 55 • Life expectancy: 64 → 66.6	• SBA: 28% → 50% • DPT3 & measles coverage > 90% • Facility delivery	• SDG 3: Healthy lives at all ages • Better equity and maternal care

Challenges

- Geographic Disparities in Health Service Access
- Frequent Stock-Outs and Weak Supply Chains
- Gaps in Health Financing and High Out-of-Pocket Costs
- Health Workforce Shortages and Maldistribution
- Weaknesses in Health Information and Data Use

Lessons

- Community Engagement Yields Results
- Health Requires Multi-sectoral Action
- Technology Alone Is Not Enough
- Flexible, Data-Informed Planning Enhances Resilience
- Sustained Investment in Workforce Retention Matters

HSTP II

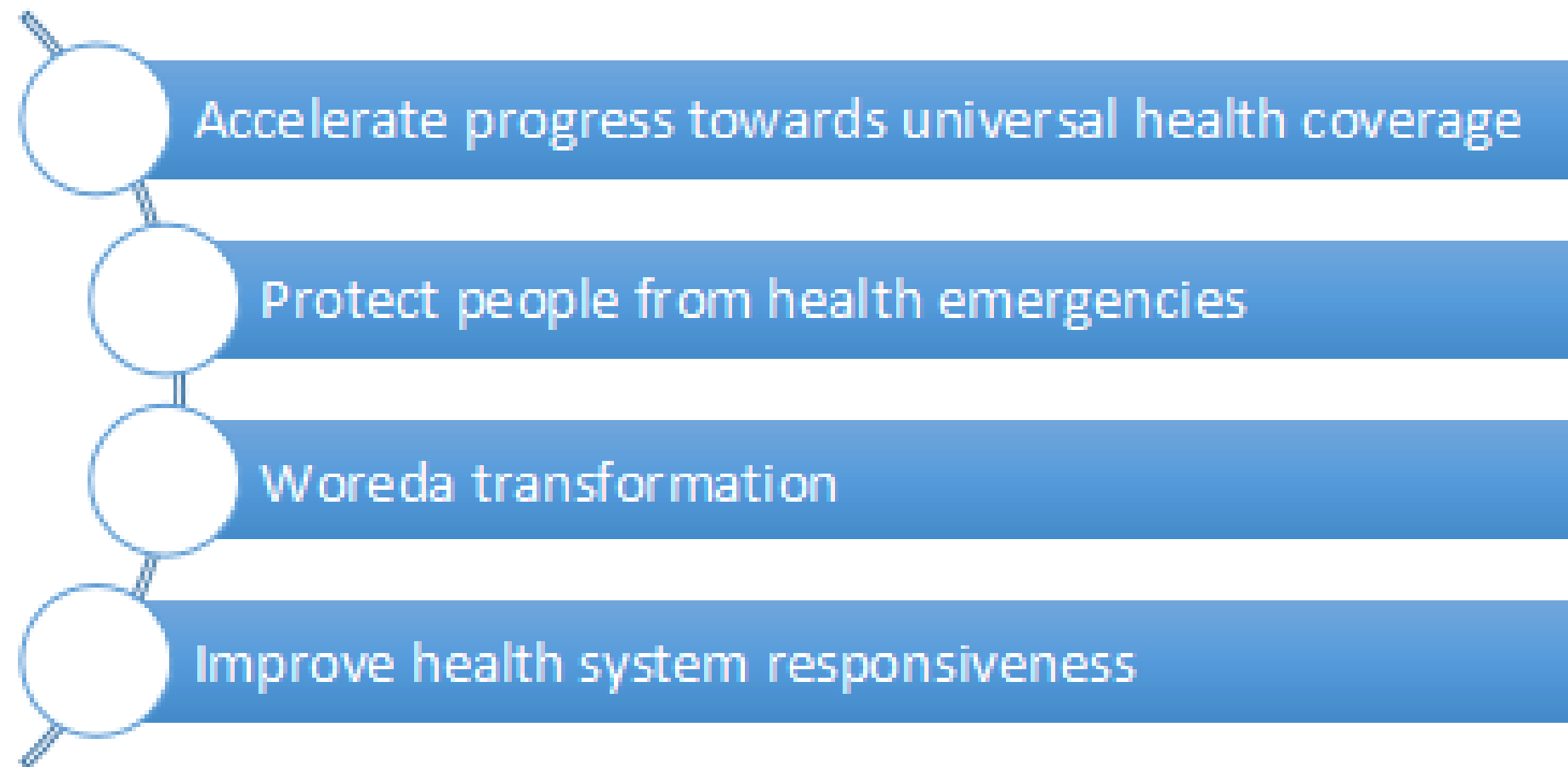
- OBJECTIVES
- PRIORITIES
- FOCUS AREAS
- STRENGTH AND LIMITATIONS



HSTP II

- HSTP II follows HSTP I during which significant improvements in Maternal Mortality, Under 5 Mortality, life expectancy, and infant mortality were seen.

Objectives of HSTP II



Targets of HSTP II

- HSTP II includes 76 measurable targets across 4 strategic pillars
- Plus additional cross-cutting targets and data monitoring systems
- 4 Strategic Pillars:
 - Quality & Equity in Health Services
 - Disease Prevention & Control
 - Health System Strengthening
 - Digital Health & Governance

Key Targets by Pillar

Quality & Equity in Health Services (24+)

- Skilled birth attendance 73 → 90%
- Stunting 38 → 25%
- Under-5 treatment coverage → 95%
- Equity in pastoral regions → 75%

Disease Prevention & Control (20+)

- HIV 95-95-95 targets
- Malaria eliminated in 50% of districts
- 50% of adults screened for NCDs annually

Health System Strengthening (18+)

- 10,000 midwives, 5,000 surgeons trained
- 90% of facilities with 24/7 water & power
- 100% primary hospitals to provide emergency surgery

Digital Health & Governance (14+)

- EHR in all referral hospitals
- Telemedicine in 50% of zones
- Annual health expenditure reviews

Cross-Cutting Targets & Tracking

- Cross-Cutting Areas
 - Nutrition: Anemia in women → 20%
 - Climate: 50% of facilities with disaster plans
 - Community: 50,000 volunteers trained
- Tracking & Data Sources
 - HMIS: Monitors service delivery
 - DHS & WHO Surveys: Validate progress

Strategic Directions of HSTP II

- Strategic directions for a policy are the high-level, overarching goals and priorities that guide the development and implementation of a specific policy.
- It includes 14 major strategic directions some of which are
 - Enhance provision of equitable and quality health services
 - Improve health emergency and disaster risk management
 - Improve human resource development and management
 - Enhance digital health technology
 - Improve health financing

Priorities

- HSTP II identifies 5 core priorities designed to build equitable, effective and resilient health system.

1. Quality & Equity

- Patient-centered, timely care for all
- Demand creation & public expectation
- Equity monitoring & tailored interventions

Cont'd

2. Information revolution

- Data culture and evidence-based decisions
- Digitization of health systems
- Stronger HIS governance

3. Motivated, Comptent and Compassionate health workforce

- Skilled, ethical, and supported staff
- Fair placement & remuneration
- Strong HR policies & enabling work environments

Cont'd

4. Health financing

- Reduce out-of-pocket expenses
- Expand health insurance
- Resource mobilization & allocation reform
- Performance-based financing

5. Leadership and Governance

- Transparent, accountable health leadership
- Stakeholder coordination & regulation
- Leadership capacity & “Health in All Policies” mainstreaming

HSTP II Key Programmatic Areas

1. RMNCAH

- Considered a priority due to high maternal & child mortality & is essential for achieving SDG 3.1 and 3.2.
- Focus Areas:
ANC, delivery, postnatal care, SBA, family planning etc...
- Key Targets:
 - MMR ↓ to 140/100,000
 - Under-5 mortality ↓ to 30/1,000
 - ↑ Modern contraceptive prevalence

Cont'd

2. Communicable Diseases (HIV, TB, Malaria).

- Considered a priority because burden for infectious disease remains high
- Focus Areas:
HIV prevention & ART, TB detection & treatment Malaria control
- Key Targets:
 - HIV incidence ↓ to 0.01%
 - TB treatment success ↑ to 95%
 - Malaria incidence ↓ by 50%

Cont'd

3. NCD's, Mental health and Injuries

- A rising cause of death and disability that's often underprioritized
- Focus Areas:
HTN and DM screening, road safety and mental health integration
- Key Targets:
 - 25% of adults screened for NCDs
 - Suicide ↓ by 10%
 - Emergency services access ↑

Cont'd

4. Emergency, Critical, & Surgical Care

- Emergency systems are weak and unequal as exposed by COVID
- There needs to be an increment in emergency medical teams & referrals as well as a strong trauma care and ambulance service.
- Targets:
 - Functional emergency units in all primary hospitals
 - Basic surgical care in general hospitals
 -

Cont'd

5. Environmental and Occupational Relief

- Poor sanitation pollution, and unsafe workplaces worsen health.
- Improvement of water resources, sanitation, air pollution...

6. Health Promotion and disease prevention

- Prevention is the cost-effective method for achieving universal healthcare
- Increasing healthy behavior and community engagement on health topics is the goal.

7. Cross-cutting issues (Gender, disability, climate change)

Strengths of HSTP II

- Commitment toward UHC
- Alignment with National and Global Frameworks
- Emphasis on Quality and Equity
- Enhance public-private partnership
- Promotes decentralized health system

Limitations of HSTP II

- Insufficient Health Budget Allocation at Federal Level
- Low Health Workforce Density & Uneven Distribution
- Weak Multi-sectoral Coordination
- Limited Progress in Public-Private Partnerships (PPP)
- Fragmented Efforts in Traditional Medicine Development
- Inadequate Digital Health Capacity & Regional Inequities
- Weak Regulation, Infrastructure, and Supply Chain Systems

Lessons for designing HSTP- III

- Shift from Service Expansion to Quality & Equity of Care
- Strengthen Health Workforce Training, Motivation & Distribution
- Enhance Decentralization with Stronger Regional Support & Accountability
- Redesign Service Delivery Architecture & Invest in PHC Infrastructure
- Boost Health Financing, PPPs & Multi-sectoral Collaboration

References

- MoH(February2021). Health sector transformation plan II. Addis Ababa, Ethiopia.
- MoH and EPHI(December2022).Health sector transformation plan I endline review study. Addis Ababa, Ethiopia.
- MoH(August 2015). Health sector transformation plan I. Addis Ababa, Ethiopia.



Thank You!